Inbrev

Sovereign inbox triage for Swiss medical practices

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Team Apertool · Hack Apertus 2026 · Track 2B — Own Project

A deeper write-up than the README: what you built, how it works, and what the numbers say.

- **Track:** Track 2B — Inbrev
- **Event:** Online
- **Team:** Apertool — Niklas Wichter
- **Demo:** [TBD: link to video]

Version of 13 October 2026. Every number is measured or cited; what is not measured yet is named as such. Measured numbers come from synthetic data whose labels have not yet been reviewed by a person.

1. Summary

Swiss general practitioners spend more and more time on administration: 9.5 hours per week in 2025, up from 8.6 in 2020, and every additional administrative hour raises the odds of stress (Zeller et al., 2026). Physicians in private practice report 54 minutes per day for requests from authorities and insurers alone (Golder et al., 2025). Much of this arrives through the practice inbox, which US studies measure as a time block of its own (85 minutes per day, 243 messages per week) (Arndt et al., 2017; Tai-Seale et al., 2019). Patient messages fall under professional secrecy, which binds physicians and their assistants alike (StGB, 1937), so sending them to a foreign cloud model is not an option.

Inbrev sorts the inbox of a Swiss practice locally with Apertus. It assigns each message to one of nine administrative routes, derives the administrative deadline and the person in charge, extracts patient and case fields, and backs every decision with a verbatim quote from the message. It never assesses clinical content: a message that may contain symptoms is excluded from automation and handed to a person, in the same queue as messages the model is unsure about, without any ranking by severity. Headline results (Apertus 8B, prompt p3; synthetic data, labels not reviewed by a person). On 180 messages Inbrev excludes 13 of 16 messages with clinical content from automation (clinical exclusion recall 0.81, 95 % CI 0.57–0.93; keyword rules 7 of 16), routes 170 of 180 correctly and routes 84 % automatically (an assistant only checks the suggestion and its quote). Because 16 cases give a wide interval, a held-out challenge set written by an open-weights model after everything was frozen adds 44 clinical messages and 39 medical traps: 8B excludes 41 of 44 (0.93, CI 0.82–0.98; flag precision 0.91), 70B 44 of 44 (CI 0.92–1.00), keyword rules 31 of 44. Two open-weights models that labelled all 367 test messages blind agree with our labels on clinical content (κ 0.97–0.98) and route (κ 0.93); scored against their labels, the ranking against the keyword rules does not change. Further checks: 40 German (GraSCCo) and 38 French (PARHAF) clinical reports written by people, not by a model, are routed right 40/40 and 37/38; on rendered fax pages read by OCR the route drops from 56 to 50 of 60 and unreadable pages go to a person. Two same-size open models (Qwen3.5-9B, Ministral 8B) in the identical pipeline do better than Apertus 8B (route 177 vs 170 of 180, clinical 16 vs 13 of 16) and than Apertus 70B (172, 15). With local Q4 weights Inbrev runs fully offline in Docker on a network without internet access (CPU, 192 s per message); the same weights run natively on the laptop's integrated GPU at 62.7 s per message. A mini PC costs about CHF 12 per 1,000 messages at 60 messages a day.

2. Architecture

Inbrev is a fixed pipeline, not an agent. Small local models are reliable on narrowly cut tasks and weak on complex ones (Xu et al., 2026), so the model does only what rules cannot: read free text in four languages and point to the passage that supports a decision.

1. **Intake.** E-mail (`.eml` or IMAP, read-only, including PDF attachments), HIN mail, fax pages as PDF or image (Tesseract OCR for German, French and Italian), web forms.
2. **Apertus with an enforced JSON schema** (Geng et al., 2023; Willard & Louf, 2023). The schema orders each quote before its decision (route quote → route, safety quote → flag, deadline quote → date). Three samples are drawn in one request.
3. **Deterministic checks per sample.** Every quote must occur in the message: verbatim, after case and whitespace folding, or at least 90 % similar to one span; the evidence shown to a person is always the original text, never the model's copy. Language models often cite incompletely (Gao et al., 2023), and Apertus showed low faithfulness in production use (Kruckenberg, 2026), so a value without a verifiable quote is dropped. Dates must match their quote; AHV numbers must occur in the text; fields that do not belong to the chosen route are removed.
4. **Rules instead of model judgement where possible.** The deadline category is computed in code from the quoted date and the time of receipt; without a quoted date or time there is no deadline. The assignee follows from the route. A safety flag always means "doctor, today".
5. **Vote and abstention.** Route and fields by majority over the samples; the safety flag is raised if any sample or a multilingual lexical net fires. Agreement across samples is used as the uncertainty signal instead of a verbalised confidence, which is overconfident (Wang et al., 2023; Xiong et al., 2024). If the samples disagree, the model marks the message as ambiguous, or no route evidence survives, Inbrev abstains: a person decides (Wen et al., 2025).
6. **Output.** A sorted inbox (web UI: "a person decides" for unsure messages and messages with clinical content, by

Phase	Dependencies
Build time	Docker images (Python, llama.cpp), Python packages, Tesseract language data, Apertus weights from Hugging Face (checksum)
Runtime	none; the model is reached over an internal Docker network without internet access. Optional: IMAP mail pick-up

time of receipt and without a ranking among them; then today, this week, no deadline), a JSON result per message following `data/schema/triage_result.schema.json`, an API under `/v1/`, and an audit log that stores a hash and the decision, not the text. Reply drafts are fixed templates for administrative routes only; no model-written text reaches patients.

Target architecture (mandatory)
Inbrev is built for **a) on-premise** and **b) air-gapped** operation; **c) a sovereign Swiss cloud** is an option for single-doctor practices.

Health data are sensitive personal data under the Swiss Data Protection Act, which also requires data protection by design and allows processing by third parties only where no duty of secrecy forbids it (DSG, 2020). A system whose data never leave the practice avoids the question. Open weights that run on the practice's own hardware also keep the model auditable (Project Apertus et al., 2025; Riedemann et al., 2024).

Offline run with local weights. `make offline-eval` (`scripts/run_offline_eval.sh`) starts llama.cpp with a community Q4_K_M GGUF of the Apertus 8B text weights (Apache-2.0; Track 2 allows GGUF builds) and Inbrev on a Docker network with `internal: true`; only an optional nginx gateway for the web UI also sits on the host network. The log records the SHA-256 of the loaded weights (equal to the Hugging Face LFS hash, revision `248ec68`), `docker inspect` of both containers, an egress test from the Inbrev container (DNS, TCP to 1.1.1.1:443 and HTTPS all fail) and a control on the default network (all succeed), memory samples and the evaluation. llama.cpp rejects `n` > 1 (HTTP 400), so Inbrev falls back to one request per sample. On a 28-W laptop CPU (Ryzen 7 PRO 8840U, Docker Desktop) the model needs 6.2 GiB RAM and Inbrev 167 MiB; results are in §5. The run is in `evidence/offline/`.

3. Use of Apertus

- **Model:** swiss-ai/Apertus-v1.5-8B for inference; Apertus-v1.5-70B only as a comparison (Project Apertus et al., 2025)
- **How it is used:** inference (classification and extraction with an enforced JSON schema, three samples per message; one on a CPU)
- **Where it runs:** development and evaluation on the Hack Apertus endpoint (OpenAI-compatible, vLLM behind a LiteLLM proxy); target deployment on local weights (llama.cpp, GGUF Q4) inside the practice, measured offline on a CPU (§2, §5)

Settings: temperature 0.7 for the three samples, at most 900 output tokens, thinking off by default because it lowers instruction following (Wiegand, 2026); thinking is measured as an ablation. The system prompt (p2; p3 since 4 October, see section 5) lists the routes with short multilingual cues, the rules for safety, deadlines and quotes, and an output format with placeholders only.

Two observations shaped the prompt. With the schema enforced but no format example, 8B looped on whitespace inside the JSON until the token limit; with a concrete example both models copied its values, so the example uses placeholders only. On the seed messages 8B often set "today" without a date, garbled capital umlauts, HTML-escaped quotes and filled fields of other routes; steps 3 and 4 handle these cases.

4. Data

All messages, people, practices and identifiers are fictional; AHV numbers have a valid format and an invalid check digit. No real patient data is used. Licence: CDLA-Permissive-2.0.

- **Seed:** 12 messages (German, French, Italian; e-mail, fax, web form) with gold labels, in the repository. Claude (`claude-opus-5-5`) wrote them one by one in an agent session, not a person; they served as the development set for the prompt and the checks.
- **Synthetic part (v0):** 180 messages. Labels are fixed by a seeded grid *before* any text exists (route, deadline, assignee, safety flag, abstain, language, channel, writing level, mixed topics, safety traps); two generator families write the text from one documented prompt, 90 messages each (Claude and GPT-5.5). Apertus neither writes nor labels data, because models recognise and prefer their own text (Panickssery et al., 2024). Distribution: German 99, French 45, Italian 27, English 9; 16 messages with clinical content that must leave automation (7 explicit, 9 mentioned in passing) and 14 traps that sound medical but need no clinical reading; 9 genuinely ambiguous messages. A validator checks schema, verbatim quotes, date consistency, invalid AHV check digits, fictitious domains and phone numbers, and near-duplicates.
- **Swiss practice set:** 30 messages (`data/ood/`), text and labels by an Antigravity agent session (Gemini, `gemini-3.8-flash`) on 3 October; real organisation names and domains were later replaced by fictitious ones (`scripts/annotate_swiss_set.py`); each row records its edits in `provenance`; the generation prompt was not recorded.
- **Open-generator set:** 42 messages from a fresh seeded grid (seed 2027, 8 with clinical content, 7 traps), text by an open-weights model, NVIDIA Nemotron 3 Ultra (NVIDIA, 2026), through OpenRouter with the same prompt minus all example values; 3 of 45 rows failed validation and were dropped. Nobody tuned prompt, checks or lexicon on it. It tests whether the results depend on the closed generators.
- **GraSCCo report set:** 40 of the 63 synthetic German clinical letters of GraSCCo v1.1 (CC BY 4.0 on Zenodo, the authors also state CC0) (Modersohn et al., 2022), verbatim (5 shortened at a paragraph end) behind a fixed

one-line cover note, as realistic long `report_received` messages (`data/grascco/`). Labels follow one rule: route `report_received`, deadline today, no clinical flag. The letters are German and Austrian, not Swiss.

- **Clinical-exclusion challenge set (held out):** 115 messages from a new seeded grid (seed 5151), 44 with clinical content (17 explicit, 27 in passing), 39 medical traps (negations, past events, chronic conditions), more typos and dialect; text by Nemotron 3 Ultra (open weights, free tier), written on 13 October after prompt, checks and lexical net were frozen (`evidence/challenge/`).
- **PARHAF report set:** 38 French clinical reports from PARHAF (fictitious cases written by resident physicians; CC BY 4.0 / Etalab 2.0), verbatim behind a fixed cover note, same rule as GraSCCo (`data/parhaf/`).
- **External datasets** (Hugging Face) were downloaded only for local stress tests, are not redistributed and give no reported number; `data/README.md` lists each source and why it is or is not used.
- **Label check without people (yet):** two open-weights models, Nemotron 3 Super and gpt-oss-120b, labelled all 367 test messages (gold, Swiss, Nemotron, challenge) blind from the written label rules, seeing only the message (`scripts/annotate_open_models.py`, USD 0.06). They are not human annotators; agreement shows that the rules can be applied consistently, not that labels are clinically right. The 36 disputed messages form the review queue for a person; `tools/review/` (blind pass, second pass after 48 h, Cohen's κ (Cohen, 1960)) is ready for that human review, which has not happened yet.

Closed models (Claude, GPT-5.5 via Codex, Gemini via Antigravity) wrote or edited test data and code during development only, and reviewed drafts. Open-weights models have three supporting roles: Nemotron 3 Ultra wrote the Nemotron, dev and challenge sets; Nemotron 3 Super and gpt-oss-120b are the second annotators; Qwen3.5-9B and Llama 3.1 8B run once in the Inbrev pipeline as a comparison. At runtime Inbrev calls only Apertus.

Synthetic data can miss the distribution of real mail (Long et al., 2024); only the GraSCCo and PARHAF reports were written by people, and they cover a single route.

5. Evaluation

The main numbers are the clinical exclusion recall (share of messages with clinical content that the safety net takes out of automation; "safety" in the tables), the recall of the deadline "today" and the share of messages that run without a person (coverage). Clinical messages are rare, so recall and precision are reported instead of accuracy (Saito & Rehmsmeier, 2015). Further metrics: route accuracy and macro-F1, deadline accuracy, field F1, evidence faithfulness (Rashkin et al., 2023), tokens, latency and cost, broken down by language, writing level, channel and generator.

Results on the synthetic set v0 (n = 180; 16 clinical cases, 50 "today" deadlines). TF-IDF uses five-fold cross-validation. Inbrev rows run the full pipeline (schema, three samples at T = 0.7, checks, vote) on the Hack Apertus endpoint, p2 on 3 October, p3 (default) on 4 October. Coverage counts messages that are neither abstained nor flagged, at most 164 of 180. "One sample, no checks" scores the first stored 8B sample as written, without checks, net, vote or abstention (`scripts/ablation_first_sample.py`).

With 95 % Wilson intervals (p2): clinical exclusion 13/16 [0.57, 0.93] for 8B and 15/16 [0.72, 0.99] for 70B (McNemar p = 0.50), against 9/16 [0.33, 0.77] for a single 8B answer without checks and 7/16 [0.23, 0.67] for the keyword rules. Three samples add two cases (11/16 without the lexical net), the net two more; it is tuned for recall, and about half of the flags on this set are false alarms from the net (15 of 28 for 8B), which costs a doctor a glance, not a missed case. Recall differs by generator (8B: 7/7 for texts by GPT-5.5, 6/9 by Claude), so the set partly measures its authors; the open-generator and challenge sets are the control. Field F1 is 0.73 for 8B and 0.83 for 70B (paired bootstrap $p < 0.001$); the checks drop 15 % of the values 8B extracted. Mean latency per message was 3.0 s (8B) and 10.7 s (70B) on the shared endpoint.

The baselines already route 79–83 % of the synthetic messages correctly, so the route alone does not justify a language model on this set. They fail where it matters: they find at most 44 % of the messages with clinical content.

Where Apertus beats the rules. Paired on the 180 messages with p2, 8B is right where the rules are wrong 27 times and the reverse 11 times (McNemar p = 0.014): it wins on insurer mail, marketing and symptoms mentioned in passing. A rules-first hybrid (rules decide when exactly one route family matches and the net is silent) needs the model for 94 of 180 messages but drops to clinical exclusion 10/16, because 5 of 16 clinical messages look like plain administration to the rules; on the challenge set it drops from 41 to 36 of 44. We therefore keep Apertus on every message (`scripts/compare_hybrid.py`).

Challenge set. The 44 clinical messages narrow the interval of the main metric: 8B 41/44, against 31/44 for the keyword rules (10 vs 0 discordant, exact McNemar p = 0.002), 70B 44/44 (8B vs 70B p = 0.25). Of 71 non-clinical messages, including 39 traps, 8B and 70B each flag 4 (precision 0.91 and 0.92). Route: 8B wins 20 vs 4 against the rules (p = 0.0015). A TF-IDF classifier cross-validated on the set itself

Setup	Route acc.	Route F1	Safety rec.	Safety prec.	Today rec.	Deadl. acc.	Coverage
Majority class	0.14	0.03	0.00	–	0.00	0.37	1.00
Keyword rules + lexical net	0.79	0.76	0.44	0.32	0.76	0.68	0.88
TF-IDF + logistic regression	0.83	0.82	0.13	1.00	0.66	0.68	0.99
8B, one sample, no checks	0.87	0.84	0.56	1.00	0.70	0.67	0.95
Inbrev 8B, p2 (schema, checks, vote)	0.88	0.85	0.81	0.46	0.86	0.82	0.81
Inbrev 70B, p2	0.91	0.89	0.94	0.50	0.94	0.85	0.79
Inbrev 8B, p3 (default)	0.94	0.94	0.81	0.46	0.88	0.84	0.84
Inbrev 70B, p3	0.96	0.96	0.94	0.50	0.94	0.87	0.82

Further sets (Inbrev 8B, p3 unless noted)	n	Route acc.	Clinical excl.	Keyword rules route / clinical
Challenge set (held out, 8B)	115	103/115 [0.83, 0.94]	**41/44 [0.82, 0.98]**	87/115 / 31/44
Challenge set, 70B	115	106/115 [0.86, 0.96]	**44/44 [0.92, 1.00]**	–
Open generator (Nemotron)	42	39/42 [0.81, 0.98]	8/8 [0.68, 1.00]	32/42 / 6/8
GraSCCo letters (German, p2)	40	40/40 [0.91, 1.00]	– (7 false flags)	40/40 / –
PARHAF reports (French)	38	37/38 [0.87, 1.00]	– (17 false flags)	3/38 / –
Rendered fax pages via OCR (gold subset)	60	50/60 [0.72, 0.91]	12/16 flagged, 13/16 out of automation	–

reaches 39/44 (an in-domain reference, not deployable). **Reports.** On GraSCCo and PARHAF the false flags come almost only from the lexical net (6 of 7; 16 of 17, mostly "urgence" in emergency department reports); Apertus itself flags 1 of 38 French reports. Each costs a doctor's glance at a report that goes to the doctor anyway; we did not retune the net on these test sets. **Fax.** 60 gold messages rendered as degraded fax pages and read by Inbrev's own Tesseract intake (character error rate 0.27, upper bound): the route falls from 56 to 50 of 60, the 3 unreadable pages abstain, and 13 of 16 clinical messages still leave automation, as on clean text (`evidence/fax/`).

Label source. Two open-weights models labelled all 367 messages of the four generated sets blind (§4). Cohen's κ with our grid labels is 0.93 for the route and 0.97–0.98 for clinical content (gold and challenge: no disagreement on clinical content), 0.60–0.90 for the deadline and 0.40–0.42 for "ambiguous". Scored against either model or the majority of three, Inbrev 8B routes 167–169 of 180 gold messages (170 against the grid) and keeps 13/16; on every set the ranking against the keyword rules is unchanged (`evidence/agreement/`).

Other open models. In the identical pipeline (prompt p3, checks, three samples) on the gold set, Qwen3.5-9B routes 177/180 and excludes 16/16 clinical messages, against 170/180 and 13/16 for Apertus 8B (route $p = 0.04$, clinical $p = 0.25$); Ministral 8B reaches the same 177/180 and 16/16 (Llama 3.1 8B could not be measured: provider rate limit; `evidence/peers/`). The prompt was developed with Apertus, which favours Apertus if anything. On this task Apertus 8B is not the strongest small open model, and Apertus 70B (172/180, 15/16) does not close the gap. We keep Apertus because its training data are open and documented and it is developed in Switzerland, which a practice can point to when it justifies its tools; Inbrev works with any OpenAI-compatible endpoint, so the model can be swapped.

Prompt p3 for "other". p3 only sharpens route boundaries ("other": copy of records, referral elsewhere, symptoms only). It was developed on a separate dev set (60 Nemotron messages; "other" p2 0/20, p3 11/20), frozen after one dev run, then run once on every test set: gold route 158 → 170/180 (McNemar $p = 0.002$; two p2 runs differ on 3 routes), no clinical case lost (`evidence/prompt-p3/`). Other numbers below use p2 unless noted.

One sample is nearly as good as three. Replaying the stored 8B run with only the first sample (checks, net and abstention kept) gives route 157/180 and clinical exclusion 12/16 against 159/180 and 13/16 (`scripts/replay_k_samples.py`). The gain over a raw answer (9/16) comes mostly from checks and net, so local deployments use one sample.

Offline on a CPU. On 30 of the 180 messages (all 16 clinical plus 14 over the routes), the offline Docker stack with Q4 weights and one sample (p2) routes 24/30 and excludes 11/16; the endpoint (BF16, one sample) 23/30 and 12/16, the keyword rules 23/30 and 7/16. Q4 and BF16 agree on the route for 28 of 30 and on the flag for 29 of 30, so quantisation costs little here. Wall time was 192 s per message (p95 417 s) on a laptop CPU shared with other work. The same llama.cpp build and weights run natively on the laptop's integrated GPU (Vulkan, no Docker Desktop VM; `scripts/run_native_llamacpp.ps1`) need 62.7 s per message (p95 85 s) with p3: route 25/30, clinical 11/16, same route as BF16 on 27 and same flag on 29 (`evidence/native/`). On the challenge set this deployable configuration (Q4, one sample) excludes 38 of 44 clinical messages and routes 99/115, against 40/44 and 104/115 for one BF16 sample ($p = 0.63$ and 0.06). These native runs measure speed and quality; the air-gap proof remains the Docker run.

Cost. From the measured time and cited prices (excl. VAT; `scripts/cost_model.py`, `evidence/offline/`): a mini PC with the same CPU draws 38–39 W under sustained load (Kennedy, 2024), so 1,000 messages need about 2.1 kWh at the CPU run's 192 s per message, CHF 0.52 at the 2026 Swiss median for small businesses (ElCom, 2026). With the box (about CHF 700, three years) and idle power, a practice with 60 messages a day pays about CHF 12 per 1,000 messages, roughly CHF 22 a month; the box would sort up to 450 messages a day. A Swiss cloud VM with 8 vCPU (CHF 0.0448 per hour (Infomaniak, 2026a)) would cost CHF 32 a month if always on. A Swiss token API at the listed Apertus 70B price (CHF 0.70 / 2.50 per million input / output tokens (Infomaniak, 2026b); 8B is not listed) would cost CHF 2.59 per 1,000 messages with the 8B token counts and CHF 3.40 with 70B. **Value.** Staff time saved is not measured. `scripts/value_model.py` makes the arithmetic explicit: measured shares from the gold run (84 % automatic, 5.0 % automatic but wrongly routed, 8.3 % false clinical flags) times assumed volumes and handling times, each with a cautious, base and favourable value (`evidence/value/`). At 60 messages a day, 60 s by hand and 15 s to check a suggestion, an MPA saves about 35 minutes a day, net CHF 462 a month after doctor time for false flags and

running cost (all assumed, not measured); the cautious case (30 messages, 30 s by hand) loses CHF 146 a month. Break-even is 27 s of manual handling per message in the base case. A timing protocol for practice interviews will replace the assumptions.

Two small sets are too small to rank systems: on the 12 seed messages 8B routes 10, the keyword rules 11; on the 30 Swiss practice messages (`data/ood/`, 4 clinical) 8B routes 24 with p3, 70B 27 (p2), the rules 25, and all take the 4 clinical cases out of automation.

Not measured yet: latency on an NVIDIA GPU (`scripts/run_native_llamacpp.ps1 -Backend cuda` is prepared) and the time to sort messages by hand versus with Inbrev.

6. Limitations

- The synthetic set partly measures its generators; the strong surface baselines suggest it is easier to separate than real mail. Messages written by people (practice staff or patients) would be the stronger test; none are in the evaluation yet (Long et al., 2024; Panickssery et al., 2024).
- No person has reviewed the labels yet. The second labels come from two open-weights models (§4, §5), which measure whether the written rules are applied consistently, not clinical correctness; one is from the same family as the generator of two sets.
- **Regulatory boundary.** Software that automatically reprioritises by clinical criticality based on patient data qualifies as medical device software (MDCG, 2025); in Switzerland classification follows Annex VIII of the EU MDR (EU MDR, 2017; MepV, 2020). Inbrev therefore never orders by severity in its default mode: the safety flag only takes a message out of automation and puts it in the same "a person decides" queue as unsure messages, ordered by time of receipt. Intended purpose: sorting administrative mail; messages with clinical content are not processed automatically. The final classification depends on the written intended purpose; this is not legal advice.
- **Optional research mode.** `--mode clinical-triage` (off by default; the app then shows a labelled research hint) adds a keyword-based hint in the style of the Emergency Severity Index to flagged messages. It is an optional research mode without an intended medical purpose and not certified; it has not been validated. Any clinical use would require an MDR/MDSW conformity assessment first. All numbers in this report use the default administrative mode.
- No measurement on real practice data; US inbox figures transfer only partly. The GraSCCo letters are real-style clinical text, but German and Austrian; the Nemotron set reuses our label grid and prompt.
- The offline numbers come from a quantised model (Q4_K_M) on a shared laptop CPU; the BF16 numbers from the hackathon endpoint. Both are reported side by side on the same messages.

7. Reproducibility

`make run` starts the web app against `LLM_BASE_URL`; `make run-local` runs llama.cpp with local weights and Inbrev on an internal network; `make offline-eval` repeats the offline run with its log; `make test` and `make offline-check` run the tests and the non-LLM evaluation with `--network none`. `scripts/agreement.py`, `value_model.py`, `compare_hybrid.py`, `replay_k_samples.py` and `cost_model.py` recompute agreement, value, rules comparison, ablation and cost from stored runs and annotations; `python -m inbrev triage|replay|eval` reproduces the numbers from the stored runs, with 95 % Wilson intervals. Grids and cross-validation are seeded. Model runs use `apertus-v1.5-8b` and `apertus-v1.5-70b` as named by the Hack Apertus endpoint (3, 4 and 13 October), which does not expose the Hugging Face revision. The generated test texts follow in the repository after human review; predictions, grids and annotations are in `evidence/`.

8. Next steps

- Five to ten interviews with practice assistants and practice owners: real volume, category mix, share of fax; a pilot in a group practice.
- HIN connection (the IMAP import is tested against a local test server only) and practice software; a data protection impact assessment template (DSG, 2020).

License

Creative Commons Attribution 4.0 (CC-BY-4.0). All Hack-Apertus projects are open-sourced. Code: Apache-2.0. Data (seed and Swiss practice messages in `data/`): CDLA-Permissive-2.0. External datasets used only for local checks are not redistributed and keep their own licences.

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